Emergency Department Note

SYNTHETIC DOCUMENT - generated for software testing, not a real patient record

Case ID	SYN-0018	**Age / Sex**	29 / F
Setting	Emergency Dept / Obstetric unit	**Primary dx**	Eclampsia at 33 weeks

Chief complaint

Generalized seizure at 33 weeks of pregnancy.

History of present illness

Primigravida with a 3-day history of frontal headache and visual disturbance; swollen ankles and face. Collapsed at home with a witnessed tonic-clonic seizure lasting about 2 minutes. No prior epilepsy.

Past history / medications

Chronic hypertension not on treatment, antenatal visits irregular. Prenatal vitamins.

Examination

BP 188/118, HR 102, afebrile. Hyperreflexia with 3 beats of clonus. Fetal heart rate 150/min. Pitting edema. No focal deficit, no neck stiffness.

Glasgow Coma Scale

Eyes open to voice. Confused and slow, disoriented to date. Obeys commands.

GCS on arrival (post-ictal)	Score	Response
Eye opening (E)	3	To voice
Verbal (V)	4	4 - Confused
Motor (M)	6	Obeys commands
TOTAL GCS		**13**

GCS at 2 h after magnesium	Score	Response
Eye opening (E)	4	Spontaneous
Verbal (V)	5	5 - Oriented
Motor (M)	6	Obeys commands
TOTAL GCS		**15**

Investigations

Urine protein 3+, platelets 98, ALT 120, creatinine 98 umol/L, LDH 520, no hemolysis smear. CT head: no hemorrhage. Obstetric ultrasound: normal liquor volume, estimated fetal weight at 15th centile.

Assessment

Eclampsia with severe hypertension and evolving HELLP-spectrum features. Post-ictal GCS 13.

Plan / disposition

Magnesium sulfate 4 g IV load then 1 g/h infusion, IV labetalol for BP, corticosteroids for fetal lung maturity. Delivery planned once stabilized. GCS 15 at 2 h, transferred to labor ward with HDU care.